

Standard Operating Procedure

Anaesthesia Care — Pre-Anaesthesia Through Recovery

«Hospital Name» — Supports COP.13.b through COP.13.j (Stop-Work)

SOP No.	[Hospital to define]	Version	1.0
Effective Date	_____	Review Frequency	Annual, or on change
Prepared By	Quality Coordinator Name: _____ Signature: _____		
Approved By	Medical Superintendent Name: _____ Signature: _____		

1. Purpose

This SOP describes how «Hospital Name» plans, delivers, and monitors anaesthesia from pre-assessment through recovery and transfer, with the anaesthesiologist present throughout.

2. Scope

Applies to general and regional anaesthesia. Local anaesthesia is outside the scope of this SOP.

3. Responsibility

Anaesthesiologist: completes every assessment, is present throughout the procedure, and applies transfer criteria at recovery.

OT/Recovery Nursing Staff: supports monitoring and documentation throughout.

4. Procedure

4.1 Pre-anaesthesia assessment and plan

1. Complete a pre-anaesthesia assessment for every patient, routine or emergency, before OT — by a qualified anaesthesiologist.
2. Review the patient's current medications and document the review.
3. Document an anaesthesia plan covering pre-medications, type/agent/dose, special requirements, and anticipated post-anaesthesia care.

4.2 Pre-induction assessment

1. Complete a pre-induction assessment immediately before induction, by an anaesthesiologist. Document any change to the plan.
2. In urgent cases, document each assessment separately even where done sequentially or simultaneously with other steps.

4.3 Consent

1. Take anaesthesia consent separately from surgical consent, obtained by the anaesthesiologist.

4.4 Intra-anaesthesia monitoring

1. Monitor continuously: temperature, heart rate, cardiac rhythm, respiratory rate, blood pressure, oxygen saturation, and ETCO₂. For regional anaesthesia, use continual qualitative observation of ventilation in place of ETCO₂.

2. The anaesthesiologist is present throughout the entire procedure — this is an absolute requirement, with no routine exception.

4.5 Post-anaesthesia monitoring and transfer

1. Monitor vitals until complete recovery, in the recovery area, OT, or ICU as clinically appropriate.
2. Apply documented transfer criteria before moving the patient out of recovery — the anaesthesiologist confirms and records this for every patient.

4.6 Documentation and safety

1. Complete the anaesthesia chart for every patient: anaesthesiologist's name, assistants' names/designations, type of anaesthesia, medications, and date/time/signature.
2. Follow the hospital's IPC guidelines for anaesthesia procedures.
3. Log every intra-operative adverse anaesthesia event against the organisation's defined list, and apply CAPA.

5. Stop-Work Authority

Do not begin anaesthesia without a completed pre-anaesthesia and pre-induction assessment.

Do not leave the patient without a present anaesthesiologist at any point during general or regional anaesthesia, except where a documented emergency requires temporary absence per the responsible anaesthesiologist's judgement.

Do not transfer a patient from recovery without documented application of the transfer criteria.

Raising an incomplete assessment or absent anaesthesiologist before proceeding is never a disciplinary matter.

6. Records

Supported by all 11 records in the COP.13 Required Records pack.

7. Related Documents

- HCO_COP_13_v2_REWRITE_DRAFT — COP.13 policy
- SOP — Procedural Sedation Administration (COP.12)

8. Review

Reviewed annually, and immediately after any intra-operative adverse anaesthesia event.

9. References

- American Society of Anesthesiologists. (2020, December 13). Standards for basic anesthetic monitoring. <https://www.asahq.org/standards-and-practice-parameters/standards-for-basic-anesthetic-monitoring> — the continuous-presence and monitoring-parameter requirements in this SOP are adapted from this standard.